Respiratory block final (year 3)

1) 5 graph (Guyton book)
2) B graph
3) Zone 1 lost ventilation
4) Leukotrienes cause bronchoconstriction
5) LD4 is inhibited by zileuton
6) Scedosporium spp... Resistance to amphotericin B
7) Aspergillus (the test name is galactomannan)
8) RSV in infant mortality
9) Omalizumab (IgE)
10) Albuterol (immediately)
11) Lama+laba (cat 15 and no hospitalization)
12) outpatient facility (not hap)
13) Entergometrien (c/i in pregnancy)
14) No change in respiratory rate in pregnancy
15) maternal polycythemia (NOT in AFE)
16) Compensated respiratory acidosis
17) Neutropliha (not related to T2 high)
18) low-severe asthma
19) Bronchial asthma
20) Exclude CF diagnosis
21) blood glucose
22) kidney (longest buffer)
23) superior laryngeal internal nerve
24) Intrapleural same as atmospheric
25) 125, 20 in emboli
26) Butarimate
27) no anticough needed
28) no antibiotics (sinusitis)
29) ambroxol (dissolve mucopolysaccharides)
30) ciprofloxacin
31) low compliance and low fev1
32) FEV1/FVC closest to normal
33) intracellular bacilli for TB
34) Aspiration: bacteroides
35) Fusarium (a hyalohyphomycosis)
36) Hyaline membranes and collapsed alveoli in IRDS
37) Type 2 CPAM
38) terminal bronchioles (most distal)
39) CF (A kid with asthma that has clubbing)
40) hypertrophy osteoarthropathy (what causes clubbing)
41) incomplete laryngeal recanalization (laryngeal web)
42) CHAOS (the xray from lecture that had a distended baby)

43) CHARGE (had an abnormal ear, and two other things)
44) Type 2 pneumocytes
45) -6 and 0 (2 probes)
46) Add low dose maintenance ICS-formeterol (smn was taking as needed saba only and started being worse)
47) Anterior ethmoidal artery (branch of ophthalmic artery)
48) Ninth intercostal space (mid axillary area where we can take fluids)
49) Move in opposed direction (flail chest)
50) Anti basement membrane (hemoptysis and hemouria)
51) Tb (aerosol)
52) Macrolides for S.pneumonia (last slide of immuno)
53) Mycoplasma (cold aggluitin)
54) S. Pneumonia
55) Left shift and high affinity (low bpg)
56) Pulmonary hypertension (atrial defect)
57) Respiratory viral infection (new risk factor for fungal)
58) more compliance at base *V/Q ratio
59) Compliance (baby who died bcuz rafius of alveoli was small)
60) 6 liter
61) 100 mL
62) 7.5
63) 3 year old brother need INH prophylaxis (from book) (cancelled qs)
64) rifampin (withdrawal symptoms)
65) pyrazinamide (gout)
66) Adenocarcinoma (removed lobe made it better)
67) bronchioalveolar cancer
68) carcinoid cancer (block bronchus)
69) isoniazid (pins and needle tingling)
70) Silica
71) Sarcoidosis
72) Asbestosis (brown with iron stuff)
73) Small cell carcinoma (cushing)
74) Squamous cell carcinoma (hypercalcemia)
75) Emphysema
76) loss of alveolar walls no fibrosis
77) Coal worker pneumoconiosis
78) black pigment
79) Chronic bronchitis
80) mucus gland hyperplasia
81) Emphysema (smoker and upper lobe)
82) XDR: rifampin, isoniazid, ciprofloxacin, and streptomycin
83) Ceftriaxone (renal impairment one)
84) vancomycin (gram positive cocci)
85) Co2 for respiratory center and O2 to remove CO (why 95% O2 and 5% CO2)

86) Albuterol, Ipratropium, O2, iv hydrocortisone
87) O2 saturation (doesn't need ABG)
88) Montelukast for physiciatry
89) Don't respond to PO2 (central related to peripheral)
90) Stagnant hypoxia (for Reynaud disease)
91) PCO2 to central chemoreceptors (why respiratory drive is decreasing)
92) What do stretch receptors do: inhibit DRG and activate VRG
93) upper part of pons (apneustic breathing)
94) Increase residual volume (age related changes)
95) Concurrent disease in host
96) Thromboembolism (hip replacement therapy)
97) EDD calculation: 6th of January 2025
98) G3P2 +1
99) EDD calculation: 24th of February 2025
100) Bicarbonate in serum